



260157517893837

Name : Mr. MANISH
 Age / Gender : 25 Year(s)/ Male
 Contact No. : +919568259784
 Address : DELHI New Delhi
 Pin code : 110091

VID No. : 260157517893837
 PID No. : P10626569123349
 Referred by : SELF
 Registered On : 09/05/2026 7:58 AM
 Collected On : 09/05/2026 7:55 AM
 Reported On : 09/05/2026 1:35 PM

CBC Haemogram

Investigation	Observed Value	Unit	Biological Reference Interval
<u>Erythrocytes</u>			
Haemoglobin (Hb)	15.4	gm/dL	13.5-18
Erythrocyte (RBC) Count	5.60	mill/cu.mm	4.7-6.0
PCV (Packed Cell Volume)	47.9	%	42-52
MCV (Mean Corpuscular Volume)	85.6	fL	78-100
MCH (Mean Corpuscular Hb)	27.5	pg	27-31
MCHC (Mean Corpuscular Hb Concn.)	32.2	gm/dL	32-36
RDW (Red Cell Distribution Width)	14.1	%	11.5-14.0
<u>Leucocytes</u>			
Total Leucocytes (WBC) Count	5300	cells/cu.mm	4000-10500
Absolute Neutrophils CountBlood	2671	cells/cu.mm	2000-7000
Absolute Lymphocyte CountBlood	2295	cells/cu.mm	1000-3000
Absolute Monocyte Count	270	cells/cu.mm	200-1000
Absolute Eosinophils CountBlood	5	cells/cu.mm	20-500
Absolute Basophils CountBlood	58	cells/cu.mm	20-100
Neutrophils	50.4	%	40-80
Lymphocytes	43.3	%	20-40
Monocytes	5.1	%	2.0-10
Eosinophils	0.1	%	1-6
Basophils	1.1	%	0-2
<u>Platelets</u>			
Platelet count	228	10 ³ /μL	150-450
MPV (Mean Platelet Volume)	10.5	fL	6-9.5
PCT (Platelet Haematocrit)	0.24	%	0.2-0.5
PDW (Platelet Distribution Width)	16.8	%	9-17

EDTA Whole Blood

- Tests done on Automated Five Part Cell Counter. (WBC, RBC Platelet count by impedance method, WBC differential by Flow cytometry technology other parameters calculated) All Abnormal Haemograms are reviewed confirmed microscopically. Differential count is based on approximately 10,000 cells.

Dr. GARIMA AGARWAL
 M.D (Pathology)
 Consultant Pathologist
 Reg No.76648

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Investigation

Observed Value

Unit

Biological Reference Interval

Fever Panel

ESR (Erythrocyte Sedimentation Rate) 4

mm/hr

<= 14

(EDTA Whole Blood)

Method: Modified Westergren

Interpretation:

- It indicates presence and intensity of an inflammatory process, never diagnostic of a specific disease. Changes are more significant than a single abnormal test.
- It is a prognostic test and used to monitor the course or response to treatment of diseases like tuberculosis, bacterial endocarditis, acute rheumatic fever, rheumatoid arthritis, SLE, Hodgkins disease, temporal arteritis, polymyalgia rheumatica.
- It is also increased in pregnancy, multiple myeloma, menstruation, and hypothyroidism.

Malaria Parasite Detection by Smear examination

(EDTA Whole Blood , Microscopy)

Malarial parasite examination

Not Seen

Not Seen

Method : Microscopy .

Interpretation:

- Parasites are usually seen during febrile episode.
- Peripheral smear has sensitivity of 86.79% and hence repeated smear examination can be required to rule out false negativity.
- Peripheral smear examination is a screening test and other methods like QBC (quantitative buffy coat), malaria antigen test and PCR should be used for confirmation especially in low parasitic index.
- Parasitic index reflects severity of infestation (parasites per 100 RBC).
- Parasitemia after adequate therapy is indicative of resistant strains.

Associated Test:

- "Fever Panel by Multiplex PCR" for early diagnosis of Dengue virus, Chikungunya virus, Salmonella spp., West Nile virus, Plasmodium spp., Rickettsia spp. and Leptospira spp.

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Sample Collected At:

East Delhi-Lab with PSC, 430, Ground Floor, Jagriti Enclave,
 Karkardooma , Delhi

Processing Location :

Processing Location Metropolis Healthcare Limited,430,Ground
 Floor,Jagriti Enclave, Karkardooma,7428569469,Delhi-110092



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Investigation**Observed Value****Unit****Biological Reference Interval****Fever Panel****Typhi Dot - IgM antibody**

Negative

Negative

(Serum Immunochromatography)

Remark :

1. Typhidot is a dot enzyme immunoassay for the rapid diagnosis of IgM antibodies against Salmonella typhi.
2. Typhidot test becomes positive within 2-3 days after infection.
3. Sensitivity of the test is 95%, as compared to Widal 75% .
4. Negative predictive value for the test is 96.1%.

Associated Test:

- "Fever Panel by Multiplex PCR" for early diagnosis of Dengue virus, Chikungunya virus, Salmonella spp., West Nile virus, Plasmodium spp., Rickettsia spp. and Leptospira spp.

Widal Test for Typhoid

(Serum Slide Agglutination)

Salmonella Typhi - 'O' Antigen

No Agglutination

No Agglutination

Salmonella Typhi - 'H' Antigen

No Agglutination

No Agglutination

Salmonella Paratyphi - 'A' - 'H'

No Agglutination

No Agglutination

Antigen**Salmomella Paratyphi 'B' - H Antigen**

No Agglutination

No Agglutination

Interpretation :

O antigen Agglutination titre	H antigen Agglutination titre	Interpretation
No agglutination	Low titres <160	Anamnestic reaction/cross reacting antibodies.
Low titres <80	No agglutination	Anamnestic reaction/cross reacting antibodies.
Low titres <80	Low titres <160	Confirm rise in titres with repeat specimen after 2-3 weeks.
>= 80	No agglutination	Suggestive of Enteric fever.
No agglutination	>= 160, any one of either SH, STA or STB antigen.	Suggestive of Enteric fever
>= 160	>= 160, any one of either SH, STA or STB antigen.	Strongly indicate Enteric fever.
Agglutination present(any titre)	Agglutination (any titre), more than one of SH, STA or STB antigen types.	Seen generally post immunization.

- For O antigen, titres of 80 or above can be significant.

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- For H antigen, titres of 160 or above are considered significant.
- Demonstration of rising titres in paired sera is confirmatory.
- SH=S.typhi, STA=S.paratyphi A, STB=S.paratyphi B.

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Routine Examination Profile - Urine

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<u>Fever Panel</u>			
<u>Routine Examination Profile - Urine</u>			
<u>General Examination</u>			
Colour	Yellow		Pale Yellow
Transparency (Appearance)	Clear		Clear
Reaction (pH)	7.0		4.5-7.0
Specific gravity	1.015		1.005-1.030
<u>Chemical Examination</u>			
Urine Protein (Albumin)	Absent		Absent
Urine Ketones (Acetone)	Absent		Absent
Urine Glucose (sugar)	Absent		Absent
Bile pigments	Absent		Absent
Bile salts	Absent		Absent
Urobilinogen	Normal		Normal
Nitrite	Negative		Negative
<u>Microscopic Examination</u>			
Red blood cells	Nil	/hpf	0-4
Pus cells (WBCs)	1-2	/hpf	0-9
Epithelial cells	0-1	/hpf	0-4
Crystals	Absent		Absent
Cast	Absent		Absent
Bacteria	Absent		Absent
Trichomonas Vaginalis	Absent		Absent
Yeast cells	Absent		Absent

Note : 1

.Chemical examination through Dipstick includes test methods as Protein (Protein Error Principle), Glucose (Glucose oxidase-Peroxidase), Ketone (Legals Test), Bilirubin (Azo- Diazo reaction),Urobilinogen (Diazonium ion Reaction) Nitrite (Griess Method). Abnormal results of chemical examination are confirmed by manual methods.

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Investigation**Observed Value****Unit****Biological Reference Interval****2**

. Negative nitrite test does not exclude the urinary tract infections, Trace proteinuria can be seen with many physiological conditions like prolonged recumbency, exercise, high protein diet. False positive reactions for bile pigments, proteins, glucose and nitrites can be caused by peroxidase like activity by disinfectants, therapeutic dyes, ascorbic acid and certain drugs.

3

.Pre-test conditions : Void first urine ,collect mid-stream urine in clean sterile container to avoid contamination with perineal, vaginal or urethral discharge.

-- End of Report --

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